## NATIONAL SOCIAL SECURITY FUND

**HUMAN RESOURCE & ADMINISTRATION DEPARTMENT**

# Ebola Virus Disease (EVD)

## Prevention and Response Protocol for the Workplace

**❖ NSSF/HRA/OHS/PR/01 ❖**

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## Acronyms and Abbreviations

| Term | Meaning |
|---|---|
| **BCP** | Business Continuity Plan |
| **CAPA** | Corrective and Preventive Action |
| **DOSHS** | Directorate of Occupational Safety and Health Services (Ministry of Labour) |
| **EVD** | Ebola Virus Disease |
| **IMS** | Integrated Management System |
| **IPC** | Infection Prevention and Control |
| **IRT** | EVD Incident Response Team |
| **KPI / KRI** | Key Performance Indicator / Key Risk Indicator |
| **MoH** | Ministry of Health |
| **OSH** | Occupational Safety and Health |
| **PPE** | Personal Protective Equipment |
| **RACI** | Responsible, Accountable, Consulted, Informed |
| **WHO** | World Health Organization |

## 1. Purpose

This Protocol sets out how the National Social Security Fund ("the Fund") will prevent, recognise early, report and safely manage suspected Ebola Virus Disease (EVD) cases and potential occupational exposures at the workplace, while keeping critical member services running.

Its specific objectives are to:

- Protect the health and safety of employees, contractors, members and visitors.
- Define clear roles, reporting lines and response timelines.
- Ensure the Fund acts on, and never substitutes for, the instructions of competent public-health authorities.
- Protect the privacy, dignity and rights of affected persons.
- Sustain critical business processes in line with the Fund's Business Continuity Plan.

## 2. Scope

This Protocol applies to all employees, contractors, consultants, interns, visitors and other persons accessing Fund premises or undertaking work on the Fund's behalf.

It covers Head Office, regional offices, branches, Huduma Centre desks, Fund-owned and leased properties, Fund vehicles, official travel and any other work-related location or activity.

## 3. Legal, Regulatory and Standards Framework

This Protocol is implemented in line with the following instruments. Where any of them is amended, the current version applies.

| Instrument | Relevance to this Protocol |
|---|---|
| **Constitution of Kenya, 2010 (Articles 31 and 43)** | Right to privacy; right to the highest attainable standard of health. |
| **Occupational Safety and Health Act, 2007** | Employer duty to provide a safe workplace, conduct risk assessments, train workers and maintain an OSH Committee. |
| **Work Injury Benefits Act, 2007** | Reporting and compensation of occupational injury and disease, including work-related infection. |
| **Public Health Act (Cap. 242)** | Notification of infectious diseases and compliance with public-health directions. |
| **Data Protection Act, 2019** | Health data is sensitive personal data; processing must be lawful, minimal and secure. |
| **Employment Act, 2007** | Non-discrimination, sick leave and fair treatment of affected employees. |
| **MoH and WHO guidance on EVD / filovirus disease** | Case definitions, IPC, PPE, referral, exposure management and contact follow-up. |
| **ISO 45001:2018** | OSH management system: hazard identification, operational control, emergency preparedness. |
| **ISO 22301:2019** | Business continuity: BIA, recovery strategies, exercising. |
| **ISO 31000:2018 and NSSF ERM Framework (NSSF/ERM/PD/03)** | Risk assessment methodology and risk reporting. |

## 4. Definitions

For purposes of this Protocol:

| Term | Definition |
|---|---|
| **Competent public-health authority** | The Ministry of Health, the County Department of Health, or a facility or team they designate for EVD assessment and response. |
| **Contact** | A person identified by the competent public-health authority as having possibly been exposed to a person with EVD. Only that authority designates contacts. |
| **Contact tracing** | The public-health process of identifying, assessing and following up persons who may have been exposed to an infected person. |
| **Decontamination** | Specialised cleaning and disinfection of an area or item that may be contaminated with infectious material, carried out by trained and authorised personnel. |
| **Designated Incident Response Lead** | The OSH Officer, or an alternate appointed in writing, who coordinates the Fund's immediate response under Section 9. |

| Term | Definition |
|---|---|
| **Ebola Virus Disease (EVD)** | A severe, often fatal infectious disease caused by orthoebolaviruses. It spreads through direct contact with the blood or body fluids of a symptomatic or deceased person, or with objects contaminated by them. It is not spread through the air in ordinary workplace settings. |
| **Infection Prevention and Control (IPC)** | Measures used to prevent or reduce the transmission of infectious agents. |
| **Isolation** | Separation of a person suspected or confirmed to have an infectious disease from others, in line with clinical and public-health requirements. |
| **Occupational exposure** | Potential work-related contact with infectious material: exposure of the eyes, mouth or other mucous membranes; contact with non-intact skin; a needlestick or other sharps injury; or unprotected direct contact with potentially infectious blood or body fluids. |
| **Personal Protective Equipment (PPE)** | Equipment worn to minimise exposure to biological hazards, selected according to the task and assessed risk. |
| **Quarantine** | Restriction of movement of a person who may have been exposed but is not unwell, as directed by the competent public-health authority. |
| **Suspected case** | A person who meets the case definition or assessment criteria in force from the competent public-health authority. The Fund does not diagnose EVD. |

## 5. Policy Statement and General Prevention Commitments

The Fund is committed to a safe and healthy workplace and to a proportionate, rights-based response to EVD. Senior Management shall resource this Protocol and the Fund shall:

1. Provide accessible handwashing facilities with soap, safe running water and other hygiene supplies (detail in Section 7.2).
2. Keep workstations, shared facilities, equipment and common areas clean and hygienic.
3. Sensitise and train employees on EVD transmission, signs and symptoms, prevention and reporting (Section 7.4).
4. Conduct workplace risk assessments and implement proportionate controls, including PPE where indicated (Section 7.1).
5. Ensure employees avoid unprotected direct contact with blood, body fluids and potentially contaminated materials.
6. Maintain and communicate current public-health information and emergency contacts (Annex A).
7. Monitor official outbreak and travel advisories and apply the alert levels in Section 6.
8. Maintain arrangements for incident reporting, medical referral, access control, environmental cleaning and waste management.

## 6. Alert Levels and Escalation

The Fund's response scales with the public-health situation. The OSH Officer recommends the alert level, the Manager, Human Resource & Administration confirms it, and the Managing Trustee/CEO approves Level 2 and Level 3. Levels are declared on official MoH or WHO information only.

| Level | Trigger (official sources only) | Minimum workplace actions |
|---|---|---|
| **0 Normal** | No active EVD outbreak in Kenya or neighbouring countries. | Routine hygiene; annual risk assessment and training; Protocol review. |
| **1 Alert** | Active outbreak in the region or a WHO Public Health Emergency of International Concern declared. | Staff awareness campaign; review travel to affected areas; verify PPE and hygiene stocks; brief IRT; confirm Annex A contacts. |
| **2 Heightened** | Confirmed EVD case(s) in Kenya. | Activate IRT; daily situation monitoring; suspend non-essential travel to affected counties; enhanced cleaning at public counters; review BCP for affected regions. |
| **3 Response** | Suspected or confirmed case, or occupational exposure, linked to Fund premises or staff. | Execute Section 9 or 10; restrict access to affected areas; liaise with public-health authority; invoke BCP for affected sites; daily reporting to Senior Management. |

## 7. Prevention and Preparedness Procedures

### 7.1 Workplace Risk Assessment

The OSH Officer shall conduct and document a workplace EVD risk assessment using the NSSF 5×5 risk matrix (NSSF ERM Framework) at least annually, after significant operational changes, and whenever the alert level rises.

The assessment shall consider:

- The nature and location of Fund operations, including high-footfall member service counters.
- Employee travel and other work-related exposure risks.
- Potential contact with blood, body fluids, biological specimens or contaminated materials.
- Cleaning, waste handling, transport and first-aid activities.
- Availability of handwashing facilities, PPE and emergency contacts.
- Business-continuity requirements and vulnerable operational processes.

Controls shall follow the hierarchy of controls (elimination, substitution, engineering, administrative, PPE) and be proportionate to the assessed risk. Residual risks rated High shall be reported to the Risk Management Department for inclusion in the departmental risk register.

### 7.2 Hygiene and Sanitation

The Fund shall:

1. Provide accessible handwashing facilities with soap and safe running water at entrances, washrooms and public counters.
2. Provide suitable hand-drying facilities and alcohol-based hand rub where appropriate.
3. Maintain clean workstations, shared equipment, washrooms and common areas.

4. Establish procedures for safe cleaning and disinfection of potentially contaminated surfaces.
5. Provide appropriate waste containers and arrangements for safe waste handling and disposal.
6. Maintain adequate stocks of cleaning materials and relevant PPE (target: at least 30 days of supply at Level 1 and above).
7. Ensure cleaning personnel receive training appropriate to their duties.

Hands visibly contaminated with blood or body fluids shall be washed promptly with soap and running water. Hand hygiene shall also be performed after removing gloves and after any possible contact with contaminated materials.

### 7.3 Personal Protective Equipment

PPE shall be selected according to the task and assessed exposure risk. Specialised EVD PPE is not routinely required for ordinary office activities where exposure to infectious material is not anticipated.

Where duties may involve exposure to potentially infectious blood or body fluids:

- A task-specific risk assessment shall determine the required PPE.
- Appropriate gloves, fluid-resistant protective clothing and eye or face protection shall be provided where indicated.
- Additional PPE shall be selected in line with current MoH and WHO guidance.
- Personnel shall receive practical training on correct donning, doffing and disposal of PPE.
- Contaminated PPE shall be handled and disposed of through approved procedures.
- PPE stocks shall be stored, inspected and replenished on a documented schedule.

> **Important:** Untrained personnel shall not provide direct care to a person suspected of having EVD or undertake specialised decontamination.

### 7.4 Staff Awareness and Training

The Fund shall provide induction and refresher training covering:

- Basic information on EVD and how it is transmitted.
- Signs and symptoms that require prompt professional medical assessment.
- Workplace reporting procedures and emergency contacts.
- Hand hygiene and general IPC measures.
- What to do when a person becomes unwell at the workplace.
- Occupational exposure reporting.
- Access-control procedures.
- Confidentiality, anti-stigma principles and responsible communication.

Training shall be recorded (Annex E) and updated whenever workplace risks or public-health guidance change significantly.

### 7.5 Travel and Work-Related Activities

Before official travel to an area affected by an EVD outbreak, the line manager and OSH Officer shall review current official travel-health advice and assess the purpose of the assignment and its exposure potential. Non-essential travel to affected areas shall be deferred at Level 2 and above.

Travelling employees shall receive relevant public-health information and be instructed to avoid direct contact with symptomatic persons, their blood or body fluids, and potentially contaminated materials.

Where a potential exposure occurs during or after official travel, the employee shall promptly contact the designated occupational-health or public-health service for assessment. Monitoring, work restrictions and follow-up shall follow current public-health guidance.

### 7.6 Preparedness Resources

The Fund shall keep the following readily accessible at every site:

- Current emergency contact details (Annex A), displayed at reception and security desks.
- Contacts for the competent public-health authority and designated referral facilities.
- Names of designated incident-response personnel and alternates.
- Current environmental cleaning and waste-management arrangements.
- Appropriate PPE and hygiene supplies.
- Training and drill records.
- Approved incident-reporting forms (Annex C).

## 8. Recognition and Reporting of Suspected Illness

Possible signs and symptoms of EVD include fever, severe fatigue, headache, muscle pain and sore throat. These may be followed by vomiting, diarrhoea, abdominal pain and, in some cases, unexplained bleeding.

> **Note:** These symptoms are not specific to EVD and are common to illnesses such as malaria and typhoid. EVD cannot be diagnosed on symptoms alone; professional medical assessment and laboratory testing are required.

An employee who develops concerning symptoms, or believes they may have had a relevant exposure, shall promptly seek professional medical advice, disclose relevant exposure information to the assessing health professional, inform their supervisor or the OSH Officer, and follow official public-health instructions.

Where a supervisor becomes aware of a potentially relevant illness, the supervisor shall:

1. Notify the Designated Incident Response Lead immediately.
2. Avoid conducting an informal or intrusive medical assessment.
3. Facilitate discreet access to professional medical assessment.
4. Minimise unnecessary close contact with the symptomatic person pending professional guidance.
5. Protect the individual's privacy and confidentiality.
6. Ensure the incident is recorded through the approved confidential reporting process.

The Fund shall apply the case definition and assessment procedures issued by the competent national or county public-health authority, and shall not classify a person as having EVD solely on the basis of symptoms, travel history or unverified information.

# 9. Response to a Suspected Case at the Workplace

## 9.1 Immediate Response

Where a person at the workplace is suspected of having EVD, the following steps apply. Timelines run from the moment the concern is first raised.

| Step | Action | Responsible | Target time |
|---|---|---|---|
| **1 Notify** | Notify the Designated Incident Response Lead and contact the competent public-health authority or designated referral service through the emergency pathway in Annex A. | Person who identifies the concern; Supervisor | Immediately (within 15 minutes) |
| **2 Protect** | Avoid unnecessary physical contact. Keep a distance of at least 1 metre, guide the person to a designated holding room or a separate area where possible, and keep others away. | Supervisor; Security | Immediately |
| **3 Arrange assessment** | Follow instructions from qualified health professionals and the responsible public-health authority. | Incident Response Lead | Within 30 minutes |
| **4 Minimise exposure** | Only authorised personnel approach the person where necessary. Any unavoidable contact is managed with the IPC precautions and PPE set by the risk assessment. | Incident Response Lead | Until handover |
| **5 Coordinate referral** | Facilitate safe medical assessment and transfer as instructed by the health authority. Do not use Fund or private vehicles unless professionally directed. | Incident Response Lead | As directed |
| **6 Restrict access** | Cordon off the affected area and any items touched. Keep it closed pending assessment, decontamination and written clearance. | Facilities; Security | Within 1 hour |
| **7 Document** | Record essential facts, actions taken, persons present and official instructions received, using Annex C. Store securely. | Incident Response Lead; HR | Within 24 hours |
| **8 Escalate** | Brief the Manager, HR & Administration and Senior Management; declare Level 3 if confirmed by the authority. | Manager, HR & Administration | Within 2 hours |

## 9.2 Prohibited Actions

Employees shall not:

- ✖ Diagnose or attempt to treat EVD.
- ✖ Touch blood, vomit, faeces or other potentially infectious body fluids without appropriate training and protection.
- ✖ Clean a potentially contaminated area using routine housekeeping procedures before it has been assessed.
- ✖ Transport a suspected case in a private or Fund vehicle without professional direction.
- ✖ Publicly identify the affected individual or circulate photographs, confidential health information, rumours or unverified information.
- ✖ Reopen an affected area before it has been assessed and cleared in writing.

## 10. Occupational Exposure Response

A potential occupational exposure includes a needlestick or other sharps injury, exposure of the eyes, mouth or other mucous membranes to potentially infectious body fluids, contact with non-intact skin, or unprotected direct contact with potentially infectious blood or body fluids.

Any worker who experiences a potential occupational exposure shall:

1. Stop the task safely.
2. Wash exposed skin promptly with soap and water. If the eyes or other mucous membranes are exposed, flush promptly with clean water or an appropriate eyewash or irrigation solution.
3. Immediately report the incident to the supervisor and the Designated Incident Response Lead.
4. Seek urgent professional medical and public-health assessment.
5. Provide information on the nature, route, time and circumstances of the exposure.
6. Follow professional instructions on monitoring, testing, work arrangements and follow-up.

**Warning:** Bleach or other harsh chemicals shall **not** be applied to the skin or other body surfaces.

The Fund shall document reported exposures, facilitate confidential professional assessment, notify the exposure as required under the Work Injury Benefits Act, 2007 and the Occupational Safety and Health Act, 2007, and cooperate with public-health authorities on follow-up.

The Fund shall not independently impose, vary or discontinue medical monitoring, quarantine or other public-health measures.

## 11. Environmental Cleaning and Waste Management

Potentially contaminated areas and materials shall be handled only by trained and authorised personnel. The Fund shall ensure that:

- Access to potentially contaminated areas is restricted.
- Cleaning personnel receive task-specific instructions and appropriate PPE.
- Approved disinfectants are used according to manufacturer instructions and public-health guidance.
- Cleaning materials are handled and disposed of safely.
- Potentially infectious waste is segregated, contained and disposed of through licensed waste handlers.
- Sharps are disposed of in puncture-resistant sharps containers.
- Potentially contaminated linen, clothing and equipment are handled under approved procedures.
- Records of specialised cleaning, waste handling and area clearance are kept (Annex D).

Where a suspected or confirmed EVD case may have contaminated an area, decontamination shall be carried out on the instructions of the responsible public-health authority or a qualified specialist team. An area shall be reopened only on written clearance.

**Warning:** Employees shall not mix disinfectants or improvise chemical concentrations.

## 12. Contact Identification and Public-Health Coordination

The Fund shall cooperate with public-health authorities in identifying and assessing persons who may have been exposed. The Fund shall:

- Preserve relevant attendance, work-assignment, access-control and visitor records.
- Provide necessary information only through the designated liaison (the Incident Response Lead).
- Maintain confidentiality and restrict access to health-related records in line with the Data Protection Act, 2019.
- Implement official instructions on assessment, monitoring, testing or work restrictions.
- Avoid designating employees as contacts on its own or requiring unnecessary disclosure of personal information.

EVD has an incubation period of **2 to 21 days**. Where a potential exposure is identified, the responsible public-health authority determines the follow-up and monitoring arrangements.

## 13. Communication and Confidentiality

All EVD-related communication shall be issued through authorised Fund channels, coordinated by Corporate Affairs with the Manager, HR & Administration. Communication shall:

- Be accurate, timely and practical.
- Refer employees to official public-health information.
- Explain relevant workplace control measures.
- Protect confidential medical and personal information.
- Avoid speculation, blame, discrimination and stigmatisation.
- Promptly correct identified misinformation.

Only the Managing Trustee/CEO or an officer they authorise may speak to the media on EVD matters. Individual health information shall be disclosed only where necessary for legitimate healthcare, workplace safety, legal or public-health purposes.

## 14. Business Continuity

EVD-related disruption shall be managed under the Fund's Business Continuity Plan, aligned to ISO 22301:2019. The arrangements shall address:

- Temporary closure of, or restricted access to, affected sites, with member services redirected to alternate branches and digital channels.
- Remote working or temporary reassignment where operationally feasible.
- Staff shortages, cross-training and continuity of critical functions.
- Supply-chain interruption and emergency procurement of hygiene supplies and PPE.
- Alternative communication and reporting arrangements.
- Safe resumption of affected operations after written clearance.
- Employee welfare and psychosocial support.

The following critical activities are prioritised. RTOs are indicative and shall be confirmed against the current Business Impact Analysis.

| Critical activity | Indicative RTO | Continuity strategy under EVD disruption |
|---|---|---|
| **Benefits processing and payments** | 24 hours | Process from alternate branch or remotely; maintain payment authorisation controls. |
| **Contributions collection and employer services** | 48 hours | Shift to self-service portal and mobile channels; redirect walk-ins to nearest open branch. |
| **Member service counters** | 48 hours | Redirect to alternate branches, Huduma Centres, call centre and digital channels. |
| **ICT systems and data** | 4 hours | No change to DR arrangements; enable secure remote access for reassigned staff. |
| **Investment operations** | 24 hours | Remote dealing and approvals under existing delegated authority. |

Operational restrictions shall be proportionate to the assessed risk and guided by competent public-health advice.

## 15. Roles and Responsibilities

### 15.1 Senior Management

- Approve and support implementation of this Protocol.
- Provide adequate financial, human and material resources.
- Approve the incident-management structure and declare Alert Levels 2 and 3.
- Ensure compliance with applicable legal and public-health requirements.
- Authorise emergency and business-continuity arrangements.
- Review significant incidents and ensure corrective action is taken.

### 15.2 Human Resource & Administration Department (including OSH)

- Own this Protocol; communicate procedures and coordinate risk assessment, preparedness, awareness, training, drills and inspections.

- Appoint the Designated Incident Response Lead and alternate in writing, and convene the IRT.
- Ensure adequate hygiene facilities, emergency supplies, occupational-health referrals, employee welfare and psychosocial support, including return-to-work arrangements.
- Coordinate reporting, investigation and confidential documentation of exposures and incidents, and liaise with public-health authorities.
- Protect employee confidentiality, dignity and non-discrimination, and protect employees who raise genuine health or exposure concerns in good faith.
- Track corrective and preventive actions (CAPA) to closure.

### 15.3 Occupational Safety and Health Committee

- Review EVD risk assessments, inspection findings and drill reports.
- Recommend improvements to Senior Management.
- Serve as a consultation channel for employees on EVD controls.

### 15.4 Supervisors and Departmental / Branch Managers

- Ensure employees understand reporting and emergency procedures.
- Monitor compliance with workplace prevention measures.
- Promptly report suspected cases and potential exposures to the Designated Incident Response Lead.
- Prevent unauthorised access to potentially affected areas.
- Support approved temporary reassignment or other operational adjustments.

### 15.5 Employees and Contractors

- Comply with hygiene and IPC requirements.
- Promptly report relevant symptoms or potential occupational exposures.
- Avoid direct contact with another person’s blood or body fluids unless trained, equipped and authorised.
- Cooperate with authorised medical and public-health assessments.
- Attend required awareness and training programmes.
- Refrain from spreading unverified information or disclosing another person’s health information.

### 15.6 Security, Cleaning and Facilities Teams

- Follow approved access-control, cleaning, waste-management and emergency procedures.
- Promptly report suspected cases or potential exposures.
- Restrict access to affected areas as instructed.
- Use PPE where required by the assessed risk and task.
- Undertake specialised EVD decontamination only when trained, equipped and authorised.

### 15.7 Risk Management Department (second line)

- Provide the risk assessment methodology and challenge the EVD risk assessment.
- Include material EVD risks and KRI breaches in quarterly risk reports to the Board Audit & Risk Committee.
- Ensure EVD scenarios are reflected in the BIA and BCP exercise programme.

### 15.8 Internal Audit (third line)

- Provide independent assurance on the design and operating effectiveness of this Protocol as part of the risk-based audit plan.

### 15.9 RACI Summary

R = Responsible, A = Accountable, C = Consulted, I = Informed.

| Activity | Senior Mgmt | HR & A / OSH | Line Managers | Facilities & Security | Risk Mgmt | Internal Audit |
|---|---|---|---|---|---|---|
| **Approve and resource Protocol** | A | R | I | I | C | I |
| **Workplace risk assessment** | I | A/R | C | C | C | I |
| **Declare alert level** | A | R | I | I | C | I |
| **Training and drills** | I | A/R | R | R | C | I |
| **Suspected case response** | I | A/R | R | R | I | I |
| **Decontamination and clearance** | I | A | I | R | I | I |
| **Business continuity invocation** | A | R | R | R | C | I |
| **KPI/KRI monitoring and reporting** | I | R | C | C | A | I |
| **Independent assurance** | I | C | C | C | C | A/R |

## 16. Training, Drills and Competency

The OSH Officer shall coordinate at least one EVD preparedness exercise per year (tabletop or simulation), and an additional exercise when Level 1 is declared. Exercises may cover:

- Reporting a suspected case.
- Activating incident-response arrangements.
- Restricting access to an affected area.
- Arranging professional medical referral.
- Responding to a simulated occupational exposure.
- Testing communication and business-continuity arrangements.

Personnel with specialised responsibilities shall receive role-specific training, including practical PPE training where their duties require it.

Exercises shall be documented (Annex E). Each identified gap shall be assigned a corrective action, owner and completion date, and tracked to closure.

## 17. Monitoring, Key Indicators and Review

The OSH Officer shall monitor the indicators below and report them quarterly to the Manager, HR & Administration and the Risk Management Department. Amber or Red results require a corrective action plan within 10 working days.

| Indicator | Type | Green | Amber | Red | Frequency |
|---|---|---|---|---|---|
| Employees trained on EVD awareness (% of headcount) | KPI | ≥ 90% | 75 to 89% | < 75% | Quarterly |
| Sites with functional handwashing at entrance and counters | KPI | 100% | 95 to 99% | < 95% | Monthly |
| PPE and hygiene stock cover (days), Level 1 and above | KRI | ≥ 30 | 15 to 29 | < 15 | Monthly |
| Exposures / suspected cases reported within 1 hour (%) | KRI | 100% | 90 to 99% | < 90% | Per incident |
| Emergency contact list (Annex A) verified | KPI | ≤ 90 days old | 91 to 180 days | > 180 days | Quarterly |
| EVD drill completed in last 12 months | KPI | Yes | Overdue ≤ 3 months | Overdue > 3 months | Annually |
| Drill and incident corrective actions overdue | KRI | 0 | 1 to 2 | ≥ 3 | Quarterly |

The OSH Officer shall also confirm continued alignment with current MoH and WHO guidance.

This Protocol shall be reviewed **at least annually**, and also after a significant workplace incident, a material change in workplace risk, or an update to relevant public-health guidance.

## 18. Alignment with Public-Health Guidance

This Protocol supports the Fund's workplace preparedness and response arrangements and shall be implemented in line with applicable MoH, national public-health and WHO guidance on IPC, early recognition, PPE, safe referral, occupational exposure management and public-health coordination.

Where current instructions from a competent public-health authority differ from this Protocol, the official requirements prevail.

This Protocol is an internal workplace document and does not constitute certification or approval by the Ministry of Health or WHO.

## 19. Approval and Implementation

This Protocol takes effect on the date of approval by the Managing Trustee/CEO.

All Heads of Department, Regional Managers and Branch Managers shall ensure that relevant employees and contractors are informed of its requirements, receive appropriate guidance or training within 60 days of approval, and understand their responsibilities.

## Annex A: Emergency Contacts

To be completed by each site and verified quarterly. Display at reception and security desks.

| Role / Service | Name | Phone | Alternate |
|---|---|---|---|
| **Designated Incident Response Lead (OSH Officer)** | | | |
| **Manager, Human Resource & Administration** | | | |
| **Branch / Regional Manager** | | | |
| **Security Supervisor (site)** | | | |
| **County Department of Health (rapid response)** | | | |
| **Ministry of Health emergency / EVD hotline** | | | |
| **Designated referral facility** | | | |
| **Ambulance service** | | | |
| **Fund medical scheme provider** | | | |
| **Specialist decontamination contractor** | | | |

## Annex B: Suspected Case Response Flow

| # | What happens | Who |
|---|---|---|
| 1 | Person becomes unwell at work with symptoms or reports possible exposure. | Any person |
| 2 | Call the Designated Incident Response Lead. Do not touch the person or their body fluids. | Witness / Supervisor |
| 3 | Guide the person to a separate area or holding room; keep others at least 1 metre away. | Supervisor / Security |
| 4 | Call the county public-health rapid response team or MoH hotline (Annex A) and follow instructions. | Incident Response Lead |
| 5 | Cordon off areas and items touched. No routine cleaning. | Facilities / Security |
| 6 | Hand over to health authority; record persons present and actions taken (Annex C). | Incident Response Lead |
| 7 | Escalate to Senior Management; consider Alert Level 3 and BCP invocation. | Manager, HR & A |
| 8 | Specialist decontamination; written clearance before reopening (Annex D). | Facilities |
| 9 | Welfare support, confidential follow-up and lessons learned. | HR & A / OSH Committee |

## Annex C: Incident and Occupational Exposure Report Form

**CONFIDENTIAL**: health information. Restrict access to HR & A and the Incident Response Lead.

| Field | Details |
|---|---|
| **Report reference no.** | |
| **Date and time of incident** | |
| **Site / location (floor, room)** | |
| **Type of incident** | Suspected case ☐ Occupational exposure ☐ Other ☐ |
| **Person affected (name, staff no. or visitor ID)** | |
| **Nature and route of exposure (if applicable)** | |
| **Immediate actions taken (with times)** | |
| **Persons present / in close proximity** | |
| **Public-health authority contacted (name, time, reference)** | |
| **Instructions received** | |
| **Areas restricted and time** | |
| **WIBA / DOSHS notification made (date)** | |
| **Reported by (name, signature, date)** | |
| **Received by Incident Response Lead (name, signature, date)** | |

## Annex D: Preparedness and Area Clearance Checklist

| # | Item | Yes / No | Comment |
|---|---|---|---|
| 1 | Handwashing stations functional at entrance, washrooms and counters | | |
| 2 | Alcohol-based hand rub available at public counters | | |
| 3 | PPE stock meets target cover (Section 7.2) | | |
| 4 | Annex A displayed and verified within 90 days | | |
| 5 | Designated holding room / area identified | | |
| 6 | Infectious waste containers and licensed handler in place | | |
| 7 | Cleaning staff trained on current procedures | | |
| 8 | Affected area decontaminated by authorised team (after incident) | | |
| 9 | Written clearance received before reopening (after incident) | | |

**Inspected by:** _____ **Signature:** _____ **Date:** _____

## Annex E: Training and Drill Log

| Date | Type (training / tabletop / simulation) | Site | No. trained | Gaps identified | Corrective action, owner, due date | Status |
|---|---|---|---|---|---|---|
| | | | | | | |
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